

RBA, Associate Analyst (RBA-C) Responsibilities, Services & Supervision Guide

V1. February 2026



This handbook exists to create *predictable, ethical, and high-quality clinical systems*. It protects clients from drift, protects staff from burnout and ambiguity, and protects the organization through clear standards, timelines, and accountability.

Why This Guide Matters

In complex clinical environments, quality does not fail loudly — it erodes quietly through unclear expectations, inconsistent supervision, delayed decision-making, and overextended clinicians. This handbook functions as a shared operating system so that quality is *designed into the system*.

Principles of ABA in Practice

Principle	What It Means	Clinical Decision-Making Example	Supervisory Behaviour Example	Personal Decision-Making Example
Generality	Skills should maintain across time, settings, and people.	If a child labels colors in therapy but not at school, the goal is not mastered.	Training staff in one session is not enough; we check maintenance over time.	If a productivity strategy works only on “good days,” it’s not truly effective.
Effective	Interventions must produce meaningful change.	A behaviour plan reducing aggression by 10% may not be clinically meaningful.	Supervision should result in observable skill improvement, not just discussion.	If a habit doesn’t meaningfully improve your life, adjust it.
Technological	Procedures must be clearly defined and replicable.	A BSP must be written so another clinician could implement it accurately.	Feedback must be specific (“increase wait time to 5 seconds”), not vague.	Clear plans outperform “I’ll try harder.”
Applied	Targets must matter socially.	Teaching eye contact is secondary to functional communication.	Supervising documentation is important because it impacts funding and care.	Focus energy on what truly improves daily functioning.
Conceptually Systematic	Interventions must align with behavioural principles.	Reinforcement schedules match skill acquisition stage.	Feedback uses reinforcement, shaping, and prompting.	Motivation and habits follow reinforcement patterns.
Analytic	Decisions must be data-based.	We change programs based on data trends, not feelings.	Staff performance is evaluated via observable measures.	Track patterns before making major changes.

Principle	What It Means	Clinical Decision-Making Example	Supervisory Behaviour Example	Personal Decision-Making Example
Behavioral	Focus on observable behaviour, not labels.	“Hits peers during transitions” vs. “is aggressive.”	“Documentation submitted late 3 times” vs. “unreliable.”	Focus on actions, not self-criticism labels.
Determinism	Behaviour is lawful and predictable.	Escalation during transitions signals environmental triggers.	Staff errors indicate skill gaps or unclear systems.	Stress behaviours follow sleep, workload, and reinforcement patterns.
Empiricism	Use objective observation and measurement.	Collect ABC data before modifying a plan.	Use fidelity checklists during supervision.	Track habits instead of guessing progress.
Parsimony	Rule out simple explanations first.	Check sleep, illness, or schedule changes before rewriting the entire BSP.	Clarify expectations before assuming resistance.	Before assuming burnout, assess workload and rest.

Application Standard

All supervisors are expected to:

- Apply GET A CAB principles in clinical decisions
- Model scientific thinking in supervision
- Interpret staff and client behaviour through lawful behavioural patterns
- Avoid emotional or reactive decision-making
- Use parsimony before escalating complexity

Scientific thinking protects clients from unnecessary intervention, protects staff from mislabeling, and protects the organization from systemic drift.

1. Ethics & Responsibilities to Staff and Clients

Rationale: Ethical practice is the foundation of sustainable clinical care. When ethics are operationalized into daily workflows, they prevent harm, reduce burnout, and protect both families and clinicians from systemic failures such as over-billing, under-supervision, or vague accountability.

All clinical decision-making and professional conduct must align with the BACB Ethics Code, CPBAO Standards, and privacy legislation. Ethics are treated as an *active practice*, not a passive checklist.

1.1 Billable vs Non-Billable Clinical Work (Ethical Use of Time)

To protect families, staff capacity, and funding integrity, all BCBAs/RBAs must adhere to approved billable activities only.

Billable activities include: - Direct sessions (1–3 hours) - Supervisions & team meetings for one client exceeding 30 minutes - Client preparation exceeding 30 minutes per client - Permanent materials created for specific clients (30–60 minutes) - Report writing (30 minutes–2 hours) - Assessments (up to 3 hours) - Letters of recommendation (30 minutes) - Parent or caregiver meetings (30–60 minutes) - Interdisciplinary meetings with external professionals (30–60 minutes). ***Rule of thumb, if it takes longer than 15 minutes, it should be billed to a client.***

Billing must always be tied to clear clinical purpose and documentation. Over-billing or unfocused prep time is considered a quality and ethical risk.

Billable Expectations and Time Allocation

As a service-based organization, all RBAs are expected to maintain a minimum billable target of 25 hours per week. Billable time may include a combination of direct client sessions, direct and indirect supervision, parent or caregiver meetings, caregiver training, assessments, report writing, and other approved client-specific clinical activities. This structure reflects the reality of clinical practice, where high-quality services extend beyond direct therapy alone. The remaining work time is intentionally allocated to non-billable responsibilities such as administrative tasks, documentation upkeep, team meetings, preparation, and professional collaboration. This balance ensures clinicians can meet ethical, clinical, and organizational expectations without sacrificing quality, sustainability, or staff wellbeing.

1.2 Ethical Foundations

All clinical decision-making and professional conduct must align with: - BACB Ethics Code for Behavior Analysts - CPBAO Standards of Practice (where applicable) - PIPEDA / PHIPA requirements for privacy and confidentiality

Ethics are not a compliance checklist — they are an active, ongoing responsibility embedded into daily practice, supervision, and systems design.

1.3 Responsibilities to Clients & Families

BCBAs/RBAs are responsible for: - Providing services that are clinically indicated, evidence-based, and socially valid - Ensuring goals are meaningful, developmentally appropriate, and tied to functional outcomes - Avoiding over-programming or billing for services without active clinical purpose - Maintaining clear communication with caregivers regarding goals, progress, and changes - Upholding dignity, assent, and least-restrictive practices at all times

Families should never pay for sessions that do not include at least preliminary goals or active clinical intent.

1.4 Responsibilities to Staff

Clinical leadership is accountable for: - Ethical supervision and scope-appropriate task delegation - Psychological safety, clarity of expectations, and predictable systems - Providing timely feedback, modeling, and access to resources - Avoiding reinforcement of burnout, chaos, or learned helplessness

Supervision should build capacity, not dependency.

2. Intake Procedure & Timelines

Rationale: Intake is the highest-risk point for quality failure. Delayed assessments, vague goals, or services without direction undermine trust and outcomes. A predictable intake timeline ensures families receive timely, purposeful care and clinicians are never expected to “figure it out later.”

2.1 5–7 Day Assessment & Intake Standard

Our organization follows a 5–7 day assessment turnaround as a quality benchmark.

Day 1: Parent Interview / BAR (1 hr – billed) - Developmental history, strengths, concerns, routines, and priorities - Preliminary goal domains identified

Day 2–3: Direct & Indirect Assessment (1.5 hrs – billed) - Structured probes, play observation, transitions, regulation, and behaviour patterns - Assessment tools selected by age and profile (ESDM, VB-MAPP, ABLIS-R, AFLS, SRS-2, MOTAS as appropriate)

Day 3–4: Initial Program Writing (1.5–2 hrs – billed) - 2–5 functional goals written based on hours available

Day 5: BSP / BRP (if needed) + Clinical Summary (1 hr – billed)

Day 6: Internal Review with Susan (30–60 min) - Feasibility, clarity, ethics, and alignment check

Day 7: Finalization & Handoff - Programs finalized in systems - Shared with caregivers

No ongoing sessions occur without preliminary goals in place.

2.2 Intake Workflow Overview (7-Day Clinical Standard)

The goal of intake is to move from referral → service readiness efficiently and ethically.

Day 0–1: Intake & File Review - Referral received and screened - Consent forms completed - Funding source identified - Initial documents uploaded

Day 1–2: Parent Interview & Indirect Assessment - Caregiver interview - Developmental and contextual history - Initial areas of concern identified

Day 3–4: Direct Observation / Assessment - Direct observation across settings (as applicable) - Use of standardized or curriculum-based tools - Identification of developmental trajectory (ASD vs. typical)

Day 5–6: Goal Selection & Program Drafting - Draft preliminary goals - Select priority domains
- Align goals to intensity and available hours

Day 7: Initial Plan & Handoff - Goals attached to services - Therapy plan ready for implementation - Supervision plan established

Delays must be documented and escalated — intake drift is a quality risk.

3. Effective Goal Selection & Triaging

Rationale: Goals drive everything — staffing, supervision, billing, and outcomes. Poor goal selection leads to wasted hours, staff frustration, and stalled progress. Strategic triaging ensures that limited hours produce maximum functional impact.

3.1 Goal Selection Principles

Effective goals are: - Functionally meaningful - Observable and measurable - Developmentally sequenced - Feasible within allocated hours

Avoid goal saturation. More goals ≠ better outcomes.

3.2 Triaging Across Domains

Priority is typically given to: 1. Safety & regulation 2. Functional communication 3. Learning readiness & routines 4. Social engagement 5. Independence skills

Goals should be stackable and allow for generalization.

3.3 Caseload & Hour-Based Decision Making

BCBAs/RBAs must scale goals to: - Client attendance - Therapist availability - Supervision capacity

If hours are limited, reduce scope, not quality.

4. Effective Supervision & Fidelity Reviews

Rationale: Supervision is a quality control system, not a meeting. Structured supervision ensures clinical decisions are reviewed, implementation is accurate, and staff develop competence over time. Fidelity monitoring protects clients from treatment drift and staff from unclear expectations.

4.1 Caseload & Supervision Structure

Each RBA/RBA-C typically carries **25-30 clients** and formally supervises **12-15 clients per week** using a predictable rotation (Week A/B or A/B/C/D depending on scope, hours and caseload).

Minimum Quality Cadence:

- **RBA Clinical QA Review:** once per month
- **Fidelity checks with clinicians:** once per month
- **Session-level feedback and competence checks:** every session
- **Competency checks (procedural & clinical):** ongoing, embedded in sessions
- **Session and/or video review with Susan (QA Manager):** bi-weekly
- **Parent Check-In:** OAP Clients (1 Hour Monthly), Private-Pay (30mins Monthly)

Supervision includes:

- Individual clinician meetings & fidelity checks
- Client discussions for QA review
- Live or recorded session review
- Shared documentation in client-specific QA folders

Clinicians must sign off on supervision notes after direct meetings.

Sample QA Schedule

	Week A	Week B	Week C	Week D
Individual Clinician Meeting & Fidelity Check	Clinician 1	Clinician 2	Clinician 3	Clinician 4
Client to Discuss for QA with Susan	Client 1	Client 5	Client 9	Client 13
Client to Discuss for QA	Client 2	Client 6	Client 10	Client 14
Client to Discuss for QA	Client 3	Client 7	Client 11	Client 15
Client to Discuss for QA	Client 4	Client 8	Client 12	Client 16

4.2 Supervision Structure

Supervision should be:

- Planned
- Time-bound
- Goal-driven

Each supervision session should include:

- Agenda setting
- Review of data or implementation
- Feedback (behavior-specific)
- Action items

4.3 Fidelity Monitoring

Fidelity reviews assess:

- Treatment integrity
- Procedural adherence
- Data accuracy

Low fidelity is a **systems issue first**, not a staff flaw.

4.4 Feedback Culture

Feedback should be:

- Immediate when possible
- Neutral and observable
- Reinforcing effort and correction

Avoid public correction that erodes trust.

5. Using Behavioral Skills Training (BST)

Rationale: Staff do not fail because they are unmotivated — they fail because skills were never taught to fluency. BST ensures staff receive instruction, modeling, practice, and feedback so expectations are achievable and fair.

BST is the default teaching methodology for staff skill acquisition.

5.1 BST Components

1. Instruction – Clear explanation of the skill
2. Modeling – Demonstration of correct performance
3. Rehearsal – Staff practice the skill
4. Feedback – Immediate, specific feedback

All four components must be present.

5.2 Application Across Roles

BST is used for: - Program implementation - Data collection - Caregiver coaching - Supervision skills

Documentation of BST supports quality assurance and ethical supervision.

6. Continued Professional Development & Quality Assurance

Rationale: Quality assurance is preventative. When QA systems are clear and instructional, issues are identified early, staff are supported, and services remain ethical and defensible. Professional development sustains long-term clinical competence.

6.1 Roles in Quality & Operations

Susan – Clinical Supervisor, Quality Assurance Lead & Trainer - Oversees clinical quality, assessments, and ethical standards - Reviews intakes, goals, and feasibility - Leads fidelity calibration and staff training - Supports analysts with triage and complex cases

Katie – Operations & On-Site Manager (Non-Clinical) - Supports scheduling, room flow, and day-to-day clinic operations - Assists with documentation systems, trackers, and logistics - Acts as a secondary support for analysts navigating operational barriers

Clinical decisions remain with BCBAs/RBAs; operational efficiency is supported through Katie to protect clinical bandwidth.

6.2 Quality Assurance Systems

Quality is proactively monitored through: - Intake turnaround metrics - Goal appropriateness reviews - Fidelity thresholds and action plans - Supervision quality checks - File audits and documentation reviews

QA is preventative, instructional, and capacity-building — never punitive.

6.3 Professional Development Expectations

BCBAs/RBAs are expected to: - Maintain certification and licensure - Engage in reflective practice - Seek consultation when outside scope - Participate in internal training and calibration

6.4 Quality Assurance Systems

Quality is monitored through: - File audits - Fidelity checks - Intake timelines - Goal appropriateness reviews - Staff and caregiver feedback

QA is preventative, not punitive.

7. Quick Clinical Decision Rules (Quick Reference)

When to Add or Change Goals - Data plateau for 3-4 weeks (depending on number of hours) - Skill mastered across 2 settings - Change in hours, staffing, or family priorities

When to Reduce Scope - Attendance below 70% - Limited weekly hours - Staff instability

When to Escalate to Susan (QA) - Ethical uncertainty - Behaviour risk - Goal feasibility concerns - Assessment ambiguity

When to Involve Katie (Operations) - Scheduling barriers - Room/resource conflicts - System or documentation access issues

Rule of Thumb: Reduce scope before increasing complexity.

8. Quality Assurance Escalation Ladder

Level 1: Coaching & Clarification - Minor fidelity or documentation issues - Addressed via supervision and BST

Level 2: Targeted Support Plan - Repeated issues or skill gaps - Increased check-ins, modeling, and feedback

Level 3: QA Review with Susan - Persistent concerns, ethical risk, or clinical drift - Formal review and corrective guidance

Level 4: System Revision or Caseload Adjustment - Issues traced to workload, structure, or scope - Adjustments made to protect quality and staff wellbeing

Escalation is instructional, not punitive. The goal is system repair, not blame.

9. Appendix: Current Templates & Tools (See Images Attached)

Rationale: Templates reduce cognitive load, increase consistency, and prevent ethical drift.

Using shared tools ensures documentation is defensible, supervision is efficient, and clinical decisions are traceable.

- Intake & Assessment Checklist (5–7 Day Standard)
- Assessment Selection & Triage Guide (by age and profile)
- Parent Interview / BAR Template
- Program Selection Matrix
- BSP / BRP Templates
- Supervision Notes Template
- Fidelity Checklist
- BST Tracking Sheet
- QA Folder Structure Guide
- Progress Report Template/End of Block Summary
- Discharge Summary Template

Templates must be used as designed - do not reinvent systems, this is a waste of resources.

The following templates support consistency and efficiency:

- Intake Checklist
- Parent Interview Template
- Consent & Service Agreement Forms
- Assessment Summary Template
- Goal Bank & Program Selection Matrix
- Supervision Notes Template
- Fidelity Checklist
- BST Tracking Sheet
- Progress Report Template
- Discharge Summary Template

Templates are living documents and may be updated with permission and general consensus as systems evolve.

This handbook is reviewed annually and updated as clinical, ethical, or organizational standards change.

Master Client Supervision Tracker Link: https://docs.google.com/spreadsheets/d/123dv-srS_eHDeadWwgBhryBun5ZrCLC-UTfi59ICd-o/edit?gid=30842042#gid=30842042

[https://docs.google.com/spreadsheets/d/123dv-srS_ehDeadWgldhBun5rZCLC-UTiH59Cd-o/edit?gid=30842042#gid=](#)

[Mail - adina@mo...](#)
[CPO Application](#)
[ChatGPT](#)
[Adobe Acrobat Ho...](#)
[Vbmap Assessm...](#)

Master Client Supervision Tracker 2026

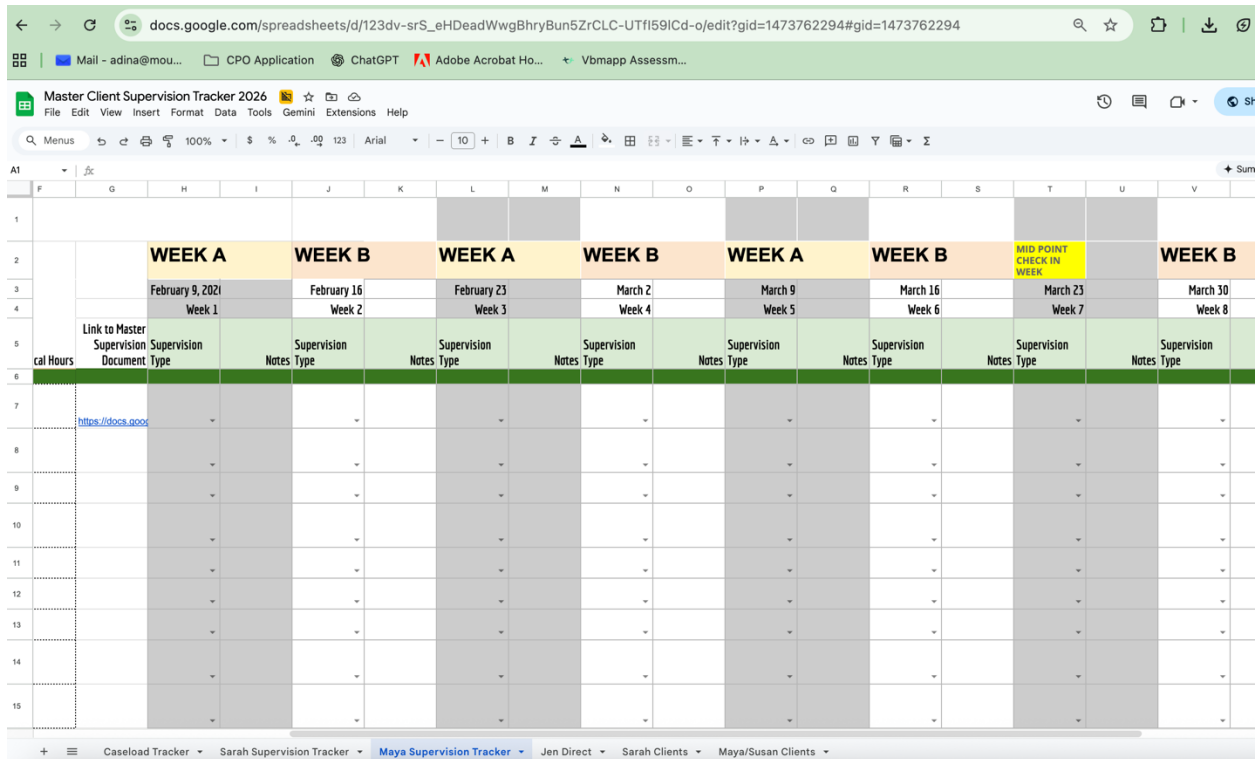
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A1	B	C	D	E	F	G	H	I	J	K	L
Clinician	Clinician Direct	Supervisor	Insurance	Client	Date of Birth	Current Age	Has BAR Been Completed	Has a Skills Completed			
Velena	SusanM	SusanM	Abinaya Thushaanth	7 years	Yes, on Jane	ABLLS-R					
Wenbo	SusanM	SusanM	Akshayen Thushaanth	9 years	Yes, on Jane	ABLLS-R					
Deila	SusanM	SusanM	Aldo Martine	4 years	Yes, on Jane	ABLLS-R					
Velena	SusanM	SusanM	Arlo Baja	7 years	Yes, on Jane	ABLLS-R					
Maya	Susan	Susan	Asia Hugh	17 years	Yes, on Jane	No, parent					
Wenbo	Susan	Susan	Benjamin Korateng	3 years	Yes, in Google Drive	VB MAPP					
Deila	Susan	Susan	Elan Mangbas	6 years	Yes, on Jane	VB MAPP					
Maya	Susan	Susan	Grayson Morant	7 years	Yes, in Google Drive	VB MAPP					
Wenbo	Susan	Susan	Heaven Smith	9 years	Yes, in Google Drive	No, parent					
Wenbo	Susan	Susan	Inshah Zahra	14 years	Yes, in Google Drive	No, parent					
Maya	Susan	Susan	Izaan Hasan	8 years	Yes, on Jane	ABLLS-R					
Deila	Susan	Susan	Jouri Smith	5 years	Yes, in Google Drive	ABLLS-R					
Maya	Susan	Susan	Judah Parlis	9 years	Yes, on Jane	ABLLS-R					
Deila	Susan	Susan	Liam Mahbobi	8 years	Yes, on Jane	ABLLS-R					
Deila	Susan	Susan	Noah Parlis	10 years	Yes, on Jane	ABLLS-R					

[Caseload Tracker](#)
[Sarah Supervision Tracker](#)
[Maya Supervision Tracker](#)
[Jen Direct](#)
[Sarah Clients](#)
[Maya/Susan Clients](#)

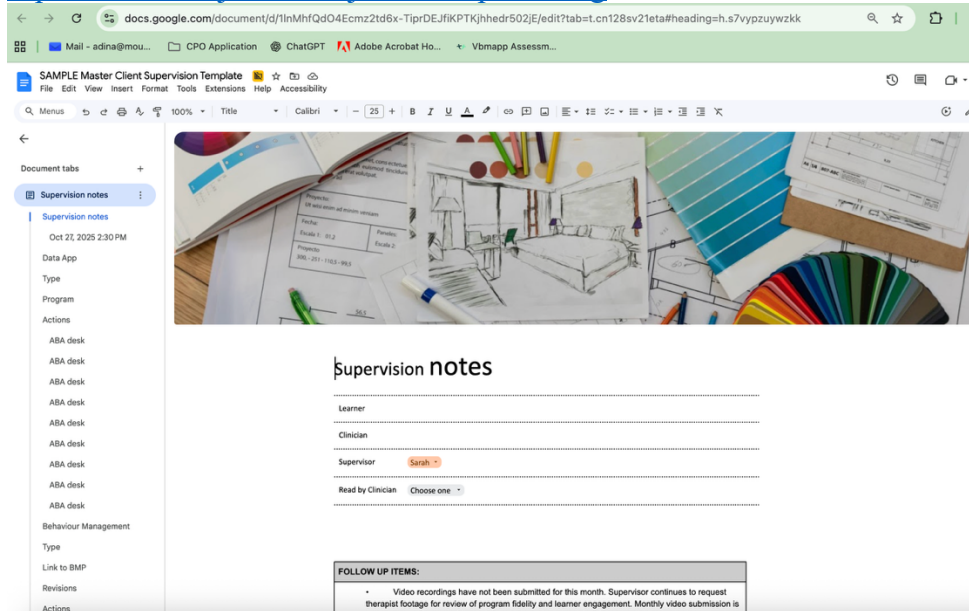
Weekly Supervision/Caseload Tracker (subsequent tabs)



Week	Supervision Type	Notes	Supervision Type	Notes	Supervision Type	Notes	Supervision Type	Notes	Supervision Type	Notes	Supervision Type	Notes	Supervision Type	Notes
February 9, 2021	Week 1		February 16	Week 2		February 23	Week 3		March 2	Week 4		March 9	Week 5	
March 16	Week 6		March 23	Week 7		March 30	Week 8							

Clinician and QA Supervision Template

Link: <https://docs.google.com/document/d/1lnMhfQdO4Ecmz2td6x-TiprDEJfIKPTKjhhdR502jE/edit?usp=sharing>



Supervision notes

Learner _____

Clinician _____

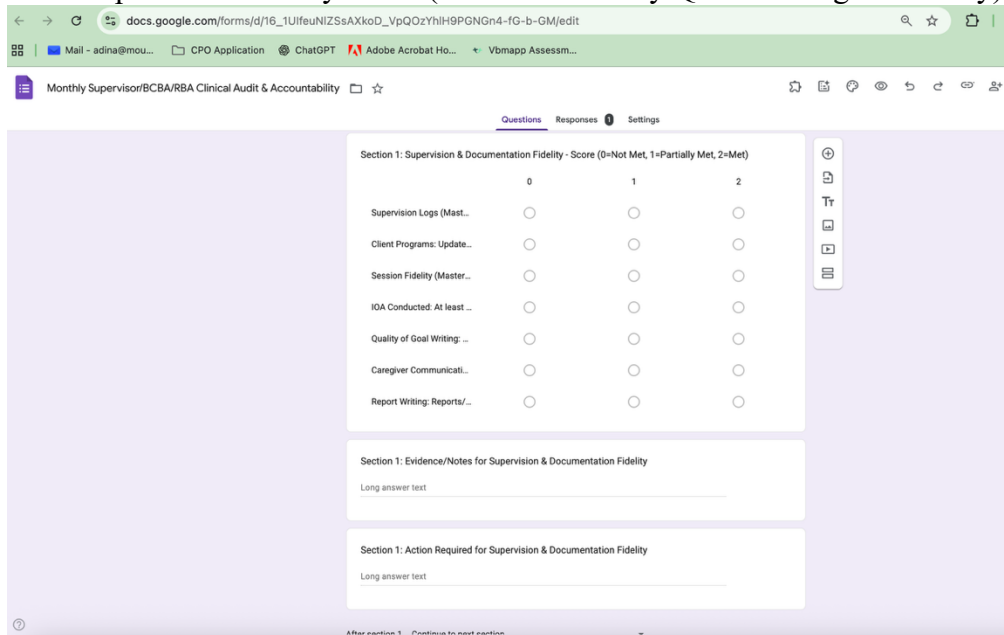
Supervisor **Sarah**

Read by Clinician **Choose one**

FOLLOW UP ITEMS:

- Video recordings have not been submitted for this month. Supervisor continues to request therapist footage for review of program fidelity and learner engagement. Monthly video submission is

RBA/Supervisor Monthly Audit (to be filled out by QA & Management only)



Section 1: Supervision & Documentation Fidelity - Score (0=Not Met, 1=Partially Met, 2=Met)

	0	1	2
Supervision Logs (Mast...	☐	☐	☐
Client Programs: Update...	☐	☐	☐
Session Fidelity (Master...	☐	☐	☐
IOA Conducted: At least ...	☐	☐	☐
Quality of Goal Writing: ...	☐	☐	☐
Caregiver Communicati...	☐	☐	☐
Report Writing: Reports/...	☐	☐	☐

Section 1: Evidence/Notes for Supervision & Documentation Fidelity

Long answer text

Section 1: Action Required for Supervision & Documentation Fidelity

Long answer text

JaneApp Family Questionnaire (Partial Bar, Auto-sends upon Intake)

These can be downloaded and uploaded to the Google Folders as well, and used as a reference guide to filling out the bar template on the Drive, as there may be missing/vague information:



Questionnaires — **Step 2 of 3**
You are completing the following intake forms: Client History (Partial BAR)

Client History (Partial BAR)

As parent or guardian, please take a few moments to help your child complete this history form prior to his/her session. An accurate health history is important to help us better understand the challenges your child may face, and ensures the delivery of safe and the best possible therapy treatment. Client information contained within this form is strictly confidential.

Health History

Caregiver's Names (Mother, Father, Primary Caregivers)*

Child's Full Name*

Child's Date of Birth*

What is the main reason for seeking therapy for your child? Include your top priorities.*

Does your child have a formal diagnosis of ASD or other?

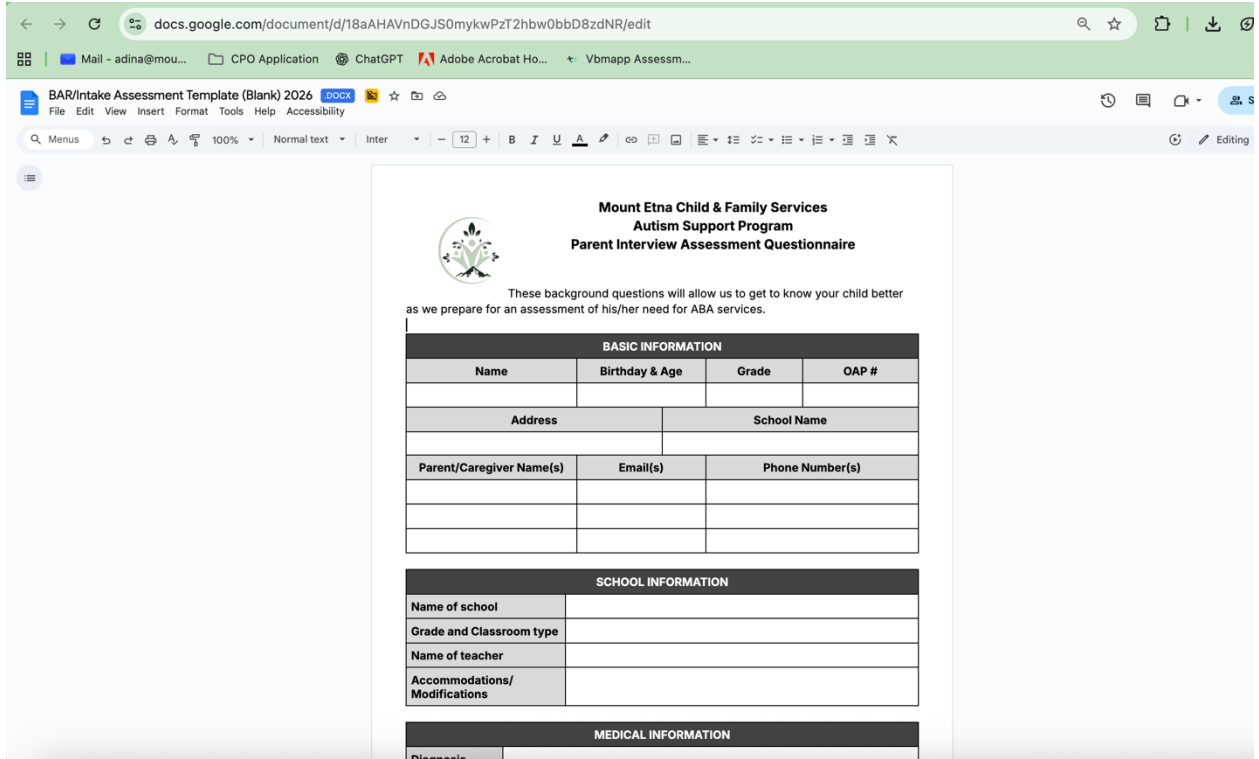
Is there any history of mental health in the family (ASD, ADHD, Bipolar, Schizophrenia)

Please check all that applies to your child's development.

Check Boxes: Areas of Needed Support

☐ Behaviour	☐ Self-Help	☐ Executive Functioning (Flexibility/Attention)
☐ Language & Communication	☐ Following Routines	☐ Early Academics
☐ Peer Interaction	☐ Play & Leisure	

BAR Template Draft (Use this to fact check and cross reference when speaking with the family):
<https://docs.google.com/document/d/18aAHAVnDGJS0mykwPzT2hbw0bbD8zdNR/edit>



**Mount Etna Child & Family Services
Autism Support Program
Parent Interview Assessment Questionnaire**

These background questions will allow us to get to know your child better as we prepare for an assessment of his/her need for ABA services.

BASIC INFORMATION			
Name	Birthday & Age	Grade	OAP #
Address		School Name	
Parent/Caregiver Name(s)	Email(s)	Phone Number(s)	

SCHOOL INFORMATION	
Name of school	
Grade and Classroom type	
Name of teacher	
Accommodations/Modifications	

MEDICAL INFORMATION	
Diagnosis	

Client Folder Layout (Each Client should have the following folders)

 Curriculum Assessments

 Interdisciplinary Reports

 Behaviour Plan

 Clinician Folder

 Materials

 Client specific feedback

 Caregiver related

 BAR/Intake Reports

 Session Videos/Quality Assurance



Initial Treatment Planning Template (On JaneApp, once QA Approves Draft) Once approved, share with parents, download and upload to individual Client Folder.

Chabise Yankov
Profile Edit / Settings **Client** Appointments Billing Communications Files Groups **New**

Assessment & Goals of Treatment

Goal of Treatment

Services will aim to improve development of **CLIENT** by systematically applying behavioural principles to bring about positive and meaningful behaviour change. These services are tailored to meet the unique needs **CLIENT**, focusing on enhancing skills and reducing challenging behaviours. The interventions are evidence-based and data-driven, promoting learning and independence across various domains such as communication, social skills, academic performance, and daily living activities.

Benefits & Risks of Treatment

Benefits:

- Acquire new skills.
- Diminish or eliminate challenging behaviors, substituting them with socially acceptable and meaningful positive behaviors.
- Enhance independence.

Risks:

- Displaying frustration or distress when encountering challenges in learning new tasks.
- Exhibiting a heightened recurrence of previously learned behaviors during intervention, prior to the replacement of these behaviors with more favorable ones.
- Reacting unfavorably to the implementation of specific rewards or other Applied Behavior Analysis (ABA) methods.

It's not possible to foresee all potential risks and benefits; the provided list is not comprehensive. Your clinical team will make a reasonable attempt to inform you of the known risks and benefits associated with each method that will be employed.

Assessment Tools & Sources

Assessment was completed with **CHILD** in group on **DATE**. Therapists assessed the following skills:

Assessment Results

Overall, **CHILD** demonstrated **#/5** of the skills assessed. The following areas are targeted for treatment based on the assessment and parent report:

Behaviour Reduction Plan

☐ Yes (Appended) ☐ No (Not Applicable)

Behaviour Service Plan

Clinical Recommendations & Rationale

During the evidence-based behavioral service period, **CHILD** continue with clinical programming as specified. Your family will play an active role in setting goals and planning interventions for your child. Utilizing the information provided by your family and the assessment results, **CHILD** will receive evidence-based behavioral services tailored to address individual needs, strengths, and objectives. If deemed suitable for your child, referrals to additional services will also be considered.

Recommendations, Duration, Intensity & Location

It is recommended that **CHILD** ADD GROUP/PROGRAM DETAILS HERE.

Intervention Plan & Goals

End of Block Summary (for Groups, can be adapted for non-groups)

Chabise Yankov
Profile Edit / Settings **Client** Appointments Billing Communications Files Groups **New**

of Sessions Attended

This report summarizes the child's progress during the referenced group ABA therapy block, including skills taught, participation observed, and progress toward group-based goals. The report is completed by a non-registered Behavioural Clinician and will be reviewed, analyzed, and approved by a Registered Behaviour Analyst (RBA) in accordance with CPBAO standards.

Summary of Progress

Group Theme/Goals

Supports Used in Groups

☐ Modeling	
☐ Prompting	
☐ Reinforcement Systems	
☐ Visual Supports	
☐ Token Systems/Group Contingencies	
☐ Behaviour Momentum (Easy-Easy-Hard)	
☐ Priming & Pre-Teaching	
☐ Peer Modeling/Video Modeling	
☐ Sensory/Flexible Seating Supports & Options	

Goals Mastered (Check)

☐ Goal 1:	
☐ Goal 2:	
☐ Goal 3:	
☐ Goal 4:	
☐ Goal 5:	

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www.mountetnachildservices.com



Master Staff Feedback Form (In each Clinician Folder)

Master Staff Feedback Form (Clinical/Non-Clinical Skills) 📄 ☆ 📁 📎

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	B	C	D	E	F	G	H	I	J	K	L	M	N	O
Supervised Clinician Feedback Form														
Instructor	Evaluate your RBT's performance and enter scores in every domain. Score 1 for correct and 0 for incorrect.													
Date	Nov. 4, 2025	January 6	January 19											
Client Initials	RV	RV	ALM											
Staff Initials	V	V	V											
Supervisor Initials	KS	KS	KS											
Organization														
Arrives on time	1	1	1											
Environment is safe with limited distractions	1	1	1											
Programs/client binder reviewed and updated														
Data sheets updated														
Mastered targets updated														
Reinforcers, data sheets, materials organized and	1	1	1											
Percent Correct	100.00%	100.00%	100.00%	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!
Professionalism														
Maintains dress code	1	1	1											
Communicates respectfully with	1	1	1											
Appropriately accepts feedback from parents	1	1	1											
Appropriately accepts feedback from	1	1	1											
Follows/implements supervisor	1	1	1											
Percent Correct	100.00%	100.00%	100.00%	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!	#DIV/0!
Program Implementation														
Starts session with	1	1	1											
Establishes instructional control	1	1	1											
Establishes attending before providing SD	0	1	1											
Confirms MO of the learner	0	0	1											
Maintains control of reinforcers	1	1	1											
Supervision Feedback Scores ▾ Task List Competency ▾ Bonus Scorecard ▾ Feedback Overview (Graphed) ▾ Data Overview (Graphed) ▾														